Early Skin-to-Skin Contact: Immediate Physiology, Not Character

In healthy newborns, early skin-to-skin contact is associated with higher breastfeeding rates and some short-term physiologic markers; GRADE is mixed, and the practice is not a lifelong character intervention.

DDERATE-BREASTFEEDING; LOW-INFANT-PHYSIOLOGY | STATUS: DRAFT | DOMAIN: BIRTH, FEEDING, REGULATION | REPLICATION

WHAT WE KNOW (Narrowest Supported Conclusion)

Early skin-to-skin contact (naked infant prone on the mother's chest soon after birth) is a bundled care practice, not a single dose of affection. In healthy, mostly term newborns, Cochrane synthesis finds more breastfeeding at one to four months and mixed, lower-certainty infant physiologic results. Kangaroo mother care for low-birth-weight infants is a different population and must not be averaged into this card.

EVIDENCE AND CAUSAL ASSESSMENT

Moore, Bergman, Anderson & Medley (2016): 38 trials with data (3,472 women); most small; none met all good-quality criteria; no trial was blinded. Comparator is usual hospital care. This is an RCT synthesis of a bundled, unblindable intervention. Benefits cannot be assigned to parental love alone when warmth, feeding support, and reduced separation travel together.

EFFECT SIZE AND PERSISTENCE

Breastfeeding maintenance is the strongest identified outcome here. Duration findings are smaller-sample and GRADE low. Infant temperature difference is statistically present and clinically trivial. Long-term neurodevelopment is not established by this review.

BIAS, HETEROGENEITY, AND CONFOUNDING

GI attenuation: n/a (immediate postpartum clinical physiology and RCT trials, not an observational phenotype).

Performance bias is structural (the intervention is visible). Small samples, co-interventions, and heterogeneous protocols. SSC is not KMC; mixing term SSC with mortality results from very-low-birth-weight kangaroo care is a translation error.

WHAT THIS DOES NOT MEAN (Strict Boundary Conditions)

Does not mean SSC guarantees bonding, breastfeeding, or cognitive advantage, or that its absence is a character injury. It does not license treating contact as mandatory moral performance.

WHAT ACTION IS REASONABLY JUSTIFIED

Make safe, supported contact available as part of essential newborn care. Moralizing contact, or treating a missed hour as a life sentence, is not justified by these trials.

VERIFIED SOURCES & DOI IDENTIFIERS

- Moore, Bergman, Anderson & Medley (2016), Cochrane Database of Systematic Reviews (PMC6464366)